POSITION STATEMENT

ETFAD/EADV Eczema Task Force 2020 Position Paper on Diagnosis and Treatment of Atopic Dermatitis in Adults and Children

Abstract

Atopic dermatitis (AD) is a highly pruritic, chronic inflammatory skin disease. The diagnosis is made using established clinical criteria. Disease activity and burden are best assessed with a composite score that evaluates both objective and subjective symptoms, such as the **SCORing Atopic Dermatitis (SCORAD)** index.

Management of AD must consider clinical and pathogenic variability, the patient's age, and the goal of flare prevention. **Basic therapy** includes universal application of hydrating and barrier-stabilizing topical treatments, as well as avoidance of specific and non-specific triggering factors.

Visible skin lesions are treated with **anti-inflammatory topical agents**, including **corticosteroids** and **calcineurin inhibitors** (tacrolimus and pimecrolimus), the latter being preferred for sensitive areas such as the face and folds. **Topical tacrolimus** and certain mid-potency corticosteroids are evidence-based options for **proactive therapy**—defined as long-term, intermittent anti-inflammatory treatment of areas prone to frequent relapses.

Systemic anti-inflammatory or immunosuppressive treatments are evolving rapidly and require careful monitoring. **Oral corticosteroids** have a largely unfavourable benefit-risk profile and should be avoided when possible. The **IL-4Rα blocker dupilumab** is a safe, effective, and licensed, though expensive, treatment option, with potential ocular side effects. Other emerging

therapies include **biologics** targeting key pathways in the atopic immune response and various **Janus kinase (JAK) inhibitors**.

Dysbiosis and skin infections can trigger disease exacerbations and may warrant adjunctive antimicrobial therapy. **Systemic antihistamines (H1-receptor blockers)** have limited efficacy in reducing AD-related itch and skin lesions.

Adjuvant therapies include **phototherapy**, preferably **narrowband UVB** or **UVA1**. **Coal tar** may be beneficial in atopic hand and foot eczema. **Dietary recommendations** should be individualized; elimination diets should only be implemented in cases of confirmed food allergy. **Allergen-specific immunotherapy** for aeroallergens may be considered in selected patients. **Psychosomatic counselling** is recommended to manage stress-induced flares.

Evidence-based **'Eczema school' educational programmes** and **therapeutic patient education** are strongly recommended for both children and adults.

Introduction and Definitions

Atopic dermatitis (AD), also known as atopic eczema, eczema, or neurodermitis, is an inflammatory, chronically relapsing, and intensely pruritic skin disorder that frequently occurs in families with a history of atopic diseases—such as AD, food allergy, bronchial asthma, or allergic rhinoconjunctivitis.

Current understanding positions AD as a **systemic Th2-driven disease**, with a high prevalence of atopic comorbidities, particularly in patients with moderate-to-severe disease. From a clinical and histopathological perspective, AD is characterized by **eosinophilic and spongiotic skin inflammation**, with distinct, age-dependent patterns in lesion distribution and morphology.

AD affects **2–10% of young adults** and up to **20% of children**, making it one of the most prevalent skin conditions worldwide. The diversity in proposed aetiological mechanisms is reflected in the various historical and regional terms used, including *neurodermitis* and *endogenous/constitutional eczema*.

Atopy is defined as a familial tendency toward hypersensitivity of the skin and mucosa to environmental antigens, typically associated with elevated **immunoglobulin E (IgE)** levels or altered pharmacological reactivity. The **ETFAD** defines atopy as the *"familial tendency to develop Th2 responses against common environmental antigens"*, encompassing both **IgE-mediated (extrinsic)** and **non-IgE-mediated (intrinsic)** subtypes of AD.

Atopic diseases are genetically linked, with a **concordance rate of 80% in monozygotic twins** compared to **30% in dizygotic twins**. Genetic polymorphisms associated with AD involve mediators of atopic inflammation across multiple chromosomes, some of which are also implicated in respiratory atopy.